NCE EXAM PREP

Practice Exam 14

Domain 2

Intake & Assessment

30 Practice Questions | 40 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 03. Use the suggested 40-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

A strong correlation between two assessment variables indicates which conclusion?

A) The variables are related, but the correlation alone does not establish that one causes the other

B) One variable necessarily causes changes in the other

C) The two variables measure exactly the same construct

Question 2

> Following a frightening assault, a client sometimes feels detached from the body and says the surroundings seem unreal. During these episodes, the client knows the sensations are internal experiences and does not believe the world has literally changed. Substance and medical causes have not yet been assessed.

Which interpretation best guides further assessment?

A) Preserved reality testing rules out any clinically significant dissociative symptoms

B) The description suggests depersonalization or derealization phenomena that require assessment of trauma, impairment, duration, and possible external causes

C) The report establishes a delusional disorder because the client describes the environment as unreal

Question 3

When selecting a norm-referenced instrument, why is the composition of its norm group important?

A) A large norm group guarantees that every individual score is valid

B) Norms eliminate the need to consider the client's language and educational experiences

C) Score comparisons are more meaningful when the norm group is relevant to the population and purpose of use

Question 4

> A client reports occasionally hearing the client's name called while falling asleep, then immediately realizes no one spoke. The experience occurs only at sleep onset, causes no impairment, and is not accompanied by daytime hallucinations, disorganization, mood episodes, or substance use.

What is the best assessment interpretation?

A) Consider a sleep-transition phenomenon and continue monitoring context rather than diagnosing a psychotic disorder from this isolated experience

B) Diagnose schizophrenia because any auditory experience is sufficient evidence of psychosis

C) Diagnose dissociative identity disorder because the voice uses the client's name

Question 5

> During a mental status examination, a client describes receiving a major award but shows little facial expression, minimal vocal variation, and almost no observable emotional response. The client reports feeling pleased internally.

Which description best characterizes the observed affect?

A) Labile

B) Euphoric

C) Blunted

Question 6

> Two family members request counseling together. One wants reconciliation after a long estrangement, while the other wants only help setting boundaries and has no intention of restoring the relationship. Each assumes the counselor will support that person's goal.

What should the counselor do before beginning conjoint services?

A) Adopt the reconciliation goal because restoring relationships is the purpose of family counseling

B) Accept both clients without discussing the incompatible expectations until conflict arises in session

C) Clarify each person's goals and the counselor's role, then determine whether there is a mutually understood and suitable basis for conjoint work

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Question 7

> A client reports nightmares and hypervigilance after an armed robbery and says alcohol is now used nightly to fall asleep. The client minimizes the drinking but has begun missing work and needs increasing amounts for the same effect.

Which assessment approach is most appropriate?

A) Evaluate trauma symptoms and alcohol use together, including their timing, interaction, impairment, and associated risks

B) Assess only trauma because the alcohol use is merely a coping strategy rather than a possible co-occurring problem

C) Defer trauma questions until the client has achieved long-term abstinence

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Question 8

> A counselor begins an intake by reading every question from a checklist without asking why the client sought help. After several minutes, the client says, “None of this gets at what is happening to me.”

What is the best adjustment?

A) Combine necessary structured questions with open-ended inquiry about the client's concerns, goals, strengths, and expectations

B) Continue the checklist without deviation because client-generated information makes intake data unreliable

C) Stop gathering structured information and immediately begin an intervention selected by the client

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Question 9

On an assessment with a mean of 100 and a standard deviation of 15, what does a score of 85 indicate?

A) The score is one standard deviation above the mean

B) The score is one standard deviation below the mean

C) The score is at the 85th percentile

Question 10

> A client with newly emerging persecutory beliefs has stopped eating and drinking because of a conviction that all food and water are poisoned. The client appears weak, has consumed almost nothing for two days, and will not consider outpatient support.

Which level-of-care decision is most appropriate?

A) Arrange immediate medical and psychiatric evaluation because impaired intake and inability to engage safely indicate urgent risk

B) Schedule a routine appointment next week to explore the symbolic meaning of the poisoning belief

C) Mail a personality inventory and wait for the results before considering medical needs

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Question 11

> Fourteen months after a spouse's death, an adult experiences intense daily yearning, difficulty accepting the death, a sense that part of the self is gone, withdrawal from meaningful activities, and marked impairment. The response substantially exceeds expected practices in the client's cultural and religious context.

Which diagnosis is best supported by the information provided?

A) Acute stress disorder

B) Separation anxiety disorder

C) Prolonged grief disorder

Question 12

A client wants to communicate with a counselor by ordinary text message and assumes messages will be read immediately at all hours. The practice uses a secure portal for routine messages, checks it only during stated business hours, and directs emergencies to designated crisis resources.

What should the counselor clarify during intake?

A) Approved communication methods, privacy risks, expected response times, and procedures for urgent needs

B) The counselor will monitor every messaging channel continuously once counseling begins

C) Digital communication is never appropriate for any administrative or clinical purpose

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Question 13

In a set of assessment scores, what does the standard deviation describe?

A) The typical spread of scores around the mean

B) The accuracy of a diagnostic cutoff

C) The percentage of items measuring the intended construct

Question 14

A client living in temporary shelters says a previous office refused intake without a permanent mailing address. The current agency permits alternate contact arrangements and does not require a fixed address for counseling eligibility.

What should the intake counselor do?

A) Explain the agency's actual eligibility policy, identify a reliable contact method with the client, and continue assessing service needs

B) Require the address of the client's most recent shelter even if the client says mail there is not secure

C) Postpone all services until the client obtains permanent housing

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Question 15

A client requests a woman counselor because discussing sexual concerns with a man would conflict with the client's religious privacy practices. The agency has women counselors available, although the first available appointment may be one week later.

What is the best intake response?

A) Challenge the request as avoidance before discussing scheduling or service options

B) Assume that every member of the client's religious community has the same preference

C) Explore the client's stated preference, explain available options and wait times, and arrange a suitable match when feasible

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Question 16

A client reports taking more prescribed alprazolam than directed, drinking alcohol most evenings, and having two recent episodes of memory loss. The client becomes shaky when attempting to reduce the medication and has not told the prescriber about the alcohol use.

What is the best assessment response?

A) Assess amounts, timing, withdrawal history, co-use risks, and current medical status, and facilitate prompt medical or substance-use evaluation

B) Recommend that the client abruptly stop both substances at home before any further assessment

C) Focus only on the original anxiety symptoms because prescribed medication use cannot constitute a substance-related concern

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Question 17

A client with limited reading proficiency signs a lengthy intake form quickly and then cannot explain the basic purpose of counseling or the stated limits of confidentiality. The client says, “I sign whatever offices give me.”

What should the counselor do?

A) Treat the signature as sufficient proof of informed consent and begin the session

B) Ask a family member to summarize the form without first asking the client's preference

C) Review the material orally in plain language, offer accessible alternatives, check understanding, and invite questions before proceeding

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Question 18

A 68-year-old client reports gradually worsening forgetfulness over 18 months, including missed bill payments and repeated questions. The client's spouse also describes reduced interest and low mood. The client takes several medications and has not had a recent medical or cognitive evaluation.

Which assessment plan is most appropriate?

A) Attribute all memory concerns to depression because low mood can affect concentration

B) Diagnose a major neurocognitive disorder solely from the spouse's report of missed bills

C) Assess cognition, mood, daily functioning, medication and medical factors, and obtain relevant collateral information before forming a diagnosis

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Question 19

> At intake, an agency invites clients to allow de-identified questionnaire data to be used in a research project. Participation is optional, declining has no effect on counseling eligibility, and the research requires a separate consent process.

What should the counselor communicate?

A) Research participation is required because the questionnaires are completed during intake

B) Refusal may reduce access to counseling even though agency policy says eligibility is unaffected

C) Counseling and research are separate choices, and the client may decline research without losing services

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Question 20

> A client obtains an extremely elevated score on an anxiety inventory but appears confused when discussing several items. The client later explains that negatively worded items were interpreted in the opposite way because of reading difficulty.

How should the counselor use the result?

A) Diagnose an anxiety disorder because numerical scores are more objective than client explanations

B) Clarify item comprehension, consider whether the administration was valid, and integrate other interview and observational data before interpreting the score

C) Discard all formal assessment methods permanently because this administration may be invalid

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Question 21

> For eight months, a client has been preoccupied with having a serious illness despite minimal physical symptoms and repeated reassuring medical evaluations. The client spends hours checking the body and researching diseases, and the anxiety causes substantial impairment.

Which diagnosis is most consistent with this information?

A) Somatic symptom disorder

B) Factitious disorder imposed on self

C) Illness anxiety disorder

Question 22

An aptitude test is administered to applicants, and scores are later compared with their job performance after one year. This procedure primarily evaluates which property?

A) Face validity

B) Predictive criterion validity

C) Internal consistency

Question 23

A client believes the person conducting the intake is a fully licensed counselor. The clinician is actually a graduate intern working under the direct supervision of a licensed counselor, and clients may request assignment to another available clinician.

What should be explained before the client agrees to services?

A) Only the intern's academic program, because supervision and alternatives are administrative details

B) Nothing about training status as long as the supervisor reviews the file

C) The intern's status and role, the supervisor's involvement, relevant confidentiality implications, and available service options

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Question 24

A client shares a mobile phone with several relatives and asks the clinic not to leave voicemail or display the clinic name in text notifications. The clinic can use neutral appointment reminders or contact the client through a secure portal.

What is the most appropriate intake action?

A) Use standard voicemail because all clients must receive identical reminder messages

B) Document the client's communication preferences, explain relevant privacy limitations, and use an available safer contact method

C) Stop all appointment reminders without discussing how the client prefers to receive scheduling information

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Question 25

A 13-year-old has significant weight loss and nutritional deficiency because many foods are avoided for their texture and smell. The adolescent expresses no fear of weight gain, no body-image disturbance, and no desire to become thinner. A medical condition does not fully explain the restriction.

Which diagnosis is most consistent with this presentation?

A) Anorexia nervosa

B) Avoidant/restrictive food intake disorder

C) Body dysmorphic disorder

Question 26

> Before a client's first appointment, a roommate sends the counselor a detailed email describing the client's behavior and asks for confirmation that the client has scheduled counseling. The client has not authorized contact with the roommate.

What is the most appropriate intake response?

A) Confirm the appointment but decline to discuss the content of future sessions

B) Avoid confirming client status, preserve the information according to applicable practice procedures, and discuss collateral involvement with the client if the intake occurs

C) Reply with the counselor's preliminary diagnosis so the roommate can provide additional observations

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Question 27

Which procedure most directly evaluates interrater reliability for an observational assessment?

A) Administering the same instrument to the same clients again two weeks later

B) Comparing scores assigned independently by two trained observers watching the same behavior

C) Correlating the assessment with a different measure of the same construct

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Question 28

> A client has persistent difficulty discarding possessions regardless of value, intense distress when attempting to discard, and severe clutter that blocks the kitchen and one exit. The accumulation is not attributable to another medical condition or to another mental disorder.

Which diagnosis is best supported by these findings?

A) Hoarding disorder

B) Obsessive-compulsive personality disorder

C) Specific phobia, situational type

Question 29

> A client with a chronic psychotic disorder is considering a voluntary outpatient program. The client can state a choice but gives inconsistent explanations of the program and how its risks and benefits apply personally. The family insists that the diagnosis itself proves incapacity.

How should the counselor assess decision-making capacity?

A) Accept the family's conclusion because a psychotic disorder automatically eliminates capacity

B) Evaluate the client's ability to understand information, appreciate its personal relevance, reason about options, and communicate a stable choice

C) Determine capacity solely from whether the client agrees with the counselor's recommendation

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Question 30

> A prospective member wants to join a closed 12-session coping-skills group but expects to miss the first four meetings because of travel. The group builds each week's exercises on material from earlier sessions, and an identical group will begin the following month.

What is the best intake decision?

A) Enroll the person now because attendance expectations should be discussed only after group begins

B) Explain the group's structure and attendance expectations and explore the later group or another suitable service

C) Deny all agency services because anticipated absences show resistance to counseling

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: A

Correct Answer: The variables are related, but the correlation alone does not establish that one causes the other

Explanation: Correlation describes the direction and strength of association between variables. It does not establish causation or prove that the variables measure an identical construct.

Question 2 — Correct: B

Correct Answer: The description suggests depersonalization or derealization phenomena that require assessment of trauma, impairment, duration, and possible external causes

Explanation: Feeling detached or unreal while retaining awareness that the experience is subjective is consistent with dissociative phenomena rather than a fixed delusion. Further assessment must still address duration, impairment, trauma context, substances, and medical explanations.

Question 3 — Correct: C

Correct Answer: Score comparisons are more meaningful when the norm group is relevant to the population and purpose of use

Explanation: Norm-referenced interpretation depends on the comparison sample and the intended use of the measure. Sample size alone does not guarantee validity, and relevant client language and educational factors still require consideration.

Question 4 — Correct: A

Correct Answer: Consider a sleep-transition phenomenon and continue monitoring context rather than diagnosing a psychotic disorder from this isolated experience

Explanation: A brief auditory experience limited to sleep onset with intact insight and no other psychotic features may be a hypnagogic phenomenon. The counselor should preserve diagnostic context rather than treating one isolated experience as sufficient for a psychotic or dissociative diagnosis.

Question 5 — Correct: C

Correct Answer: Blunted

Explanation: Blunted affect refers to a marked reduction in observable emotional expression. The client's reported internal pleasure does not make the outward affect euphoric, and no rapid shifts are described to suggest lability.

Question 6 — Correct: C

Correct Answer: Clarify each person's goals and the counselor's role, then determine whether there is a mutually understood and suitable basis for conjoint work

Explanation: Incompatible unstated expectations can undermine consent and the suitability of conjoint counseling. The counselor should not impose reconciliation or defer role clarification until conflict develops.

Question 7 — Correct: A

Correct Answer: Evaluate trauma symptoms and alcohol use together, including their timing, interaction, impairment, and associated risks

Explanation: The alcohol use shows tolerance and occupational impact and may interact with trauma symptoms in a reinforcing pattern. Assessing both conditions together supports an accurate co-occurring formulation rather than treating either as ineligible for assessment until the other resolves.

Question 8 — Correct: A

Correct Answer: Combine necessary structured questions with open-ended inquiry about the client's concerns, goals, strengths, and expectations

Explanation: Structured data can support completeness, but intake also requires the client's account and priorities. Abandoning all structure or moving immediately into intervention would leave important assessment information unaddressed.

Question 9 — Correct: B

Correct Answer: The score is one standard deviation below the mean

Explanation: A score of 85 is 15 points below a mean of 100, and the standard deviation is 15. This calculation does not identify an 85th-percentile rank.

Question 10 — Correct: A

Correct Answer: Arrange immediate medical and psychiatric evaluation because impaired intake and inability to engage safely indicate urgent risk

Explanation: Near-total refusal of food and fluids, physical weakness, and fixed poisoning beliefs create immediate medical and psychiatric concerns. Routine follow-up or delayed testing would not match the severity of impaired self-care.

Question 11 — Correct: C

Correct Answer: Prolonged grief disorder

Explanation: Persistent yearning, identity disruption, difficulty accepting the death, withdrawal, and marked impairment continue beyond 12 months and exceed cultural expectations. The duration and bereavement-specific pattern do not fit acute stress disorder or separation anxiety disorder.

Question 12 — Correct: A

Correct Answer: Approved communication methods, privacy risks, expected response times, and procedures for urgent needs

Explanation: Informed consent for digital communication should establish how messages are sent, protected, monitored, and answered. It should also prevent the client from relying on a noncontinuous channel during an emergency.

Question 13 — Correct: A

Correct Answer: The typical spread of scores around the mean

Explanation: Standard deviation summarizes how dispersed scores are around the mean. It does not establish a cutoff's accuracy or indicate the percentage of items representing a construct.

Question 14 — Correct: A

Correct Answer: Explain the agency's actual eligibility policy, identify a reliable contact method with the client, and continue assessing service needs

Explanation: A permanent address is not an agency requirement, so the prior barrier should not be reproduced. Collaborative alternate contact planning permits intake to continue while respecting the client's security concerns.

Question 15 — Correct: C

Correct Answer: Explore the client's stated preference, explain available options and wait times, and arrange a suitable match when feasible

Explanation: The client has identified a personally meaningful privacy need, and the requested option is available. The counselor should respond to the individual preference without pathologizing it or generalizing it to everyone in the same religious community.

Question 16 — Correct: A

Correct Answer: Assess amounts, timing, withdrawal history, co-use risks, and current medical status, and facilitate prompt medical or substance-use evaluation

Explanation: Escalating benzodiazepine use, alcohol co-use, blackouts, and withdrawal symptoms create significant medical risk. Abrupt unsupervised cessation may be dangerous, while ignoring prescribed medication misuse would miss the central assessment concern.

Question 17 — Correct: C

Correct Answer: Review the material orally in plain language, offer accessible alternatives, check understanding, and invite questions before proceeding

Explanation: A signature does not demonstrate meaningful understanding when the client cannot explain central terms. Adapting communication and using a comprehension check supports informed choice without transferring the decision to a family member.

Question 18 — Correct: C

Correct Answer: Assess cognition, mood, daily functioning, medication and medical factors, and obtain relevant collateral information before forming a diagnosis

Explanation: Gradual cognitive change with functional decline warrants a broad assessment that includes possible neurocognitive, mood, medication, and medical contributors. Neither low mood nor one collateral report is sufficient by itself to establish the diagnosis.

Question 19 — Correct: C

Correct Answer: Counseling and research are separate choices, and the client may decline research without losing services

Explanation: The agency's policy makes research optional and independent of counseling eligibility. Separate consent prevents the intake setting from creating a false impression that access to care depends on research participation.

Question 20 — Correct: B

Correct Answer: Clarify item comprehension, consider whether the administration was valid, and integrate other interview and observational data before interpreting the score

Explanation: Misunderstood items can invalidate the meaning of the total score, so comprehension and administration conditions require review. The result should not override contradictory information, but one flawed administration also does not justify abandoning all formal assessment.

Question 21 — Correct: C

Correct Answer: Illness anxiety disorder

Explanation: The central feature is persistent fear of serious illness despite minimal symptoms and reassuring evaluations, accompanied by excessive checking and impairment. Somatic symptom disorder instead requires distressing somatic symptoms, while no deceptive behavior is described for factitious disorder.

Question 22 — Correct: B

Correct Answer: Predictive criterion validity

Explanation: Predictive criterion validity examines how well an earlier score forecasts a later relevant outcome. Internal consistency concerns relationships among test items, while face validity concerns whether a measure appears appropriate on its surface.

Question 23 — Correct: C

Correct Answer: The intern's status and role, the supervisor's involvement, relevant confidentiality implications, and available service options

Explanation: Training status, supervision, and service alternatives are material to the client's informed decision. Supervisor review does not remove the need to disclose who will provide the service and how supervision operates.

Question 24 — Correct: B

Correct Answer: Document the client's communication preferences, explain relevant privacy limitations, and use an available safer contact method

Explanation: Intake should establish a contact method that reflects the client's privacy circumstances and the clinic's actual capabilities. A uniform voicemail rule would create avoidable disclosure risk, while eliminating reminders without collaboration may create another access barrier.

Question 25 — Correct: B

Correct Answer: Avoidant/restrictive food intake disorder

Explanation: The restriction is driven by sensory characteristics and has caused weight loss and nutritional deficiency without weight-shape concerns. That pattern supports avoidant/restrictive food intake disorder rather than anorexia nervosa.

Question 26 — Correct: B

Correct Answer: Avoid confirming client status, preserve the information according to applicable practice procedures, and discuss collateral involvement with the client if the intake occurs

Explanation: An unsolicited message does not authorize confirmation that the person is a client or permit disclosure to the sender. The counselor can handle the received information appropriately and later seek the client's perspective and permission regarding collateral involvement.

Question 27 — Correct: B

Correct Answer: Comparing scores assigned independently by two trained observers watching the same behavior

Explanation: Interrater reliability concerns agreement between evaluators observing the same target. Repeating a measure over time assesses test-retest reliability, while correlation with another construct measure addresses validity evidence.

Question 28 — Correct: A

Correct Answer: Hoarding disorder

Explanation: Persistent difficulty discarding, distress associated with discarding, and clutter that compromises living areas are defining features of hoarding disorder under the supplied exclusions. A personality style or situational fear does not account for the accumulation and blocked living space.

Question 29 — Correct: B

Correct Answer: Evaluate the client's ability to understand information, appreciate its personal relevance, reason about options, and communicate a stable choice

Explanation: Capacity is decision specific and cannot be inferred solely from a psychiatric diagnosis or disagreement with a recommendation. The assessment should examine the functional abilities required for the actual decision.

Question 30 — Correct: B

Correct Answer: Explain the group's structure and attendance expectations and explore the later group or another suitable service

Explanation: The cumulative closed-group format makes missing the first third of meetings clinically and practically relevant to suitability. Transparent discussion and an available later group address the mismatch without treating travel as resistance or denying all services.
